

The Global Burden of Mental Illness: Prevalence, Impact, and the Treatment Gap

Global data overview (2023) for public health policy analysts and mental health program planners

A data-driven overview of global mental health conditions, their disease burden, and the persistent gap between need and treatment.

Hundreds of Millions Affected: 1 in 3 Women and 1 in 5 Men Face Major D

1 in 3

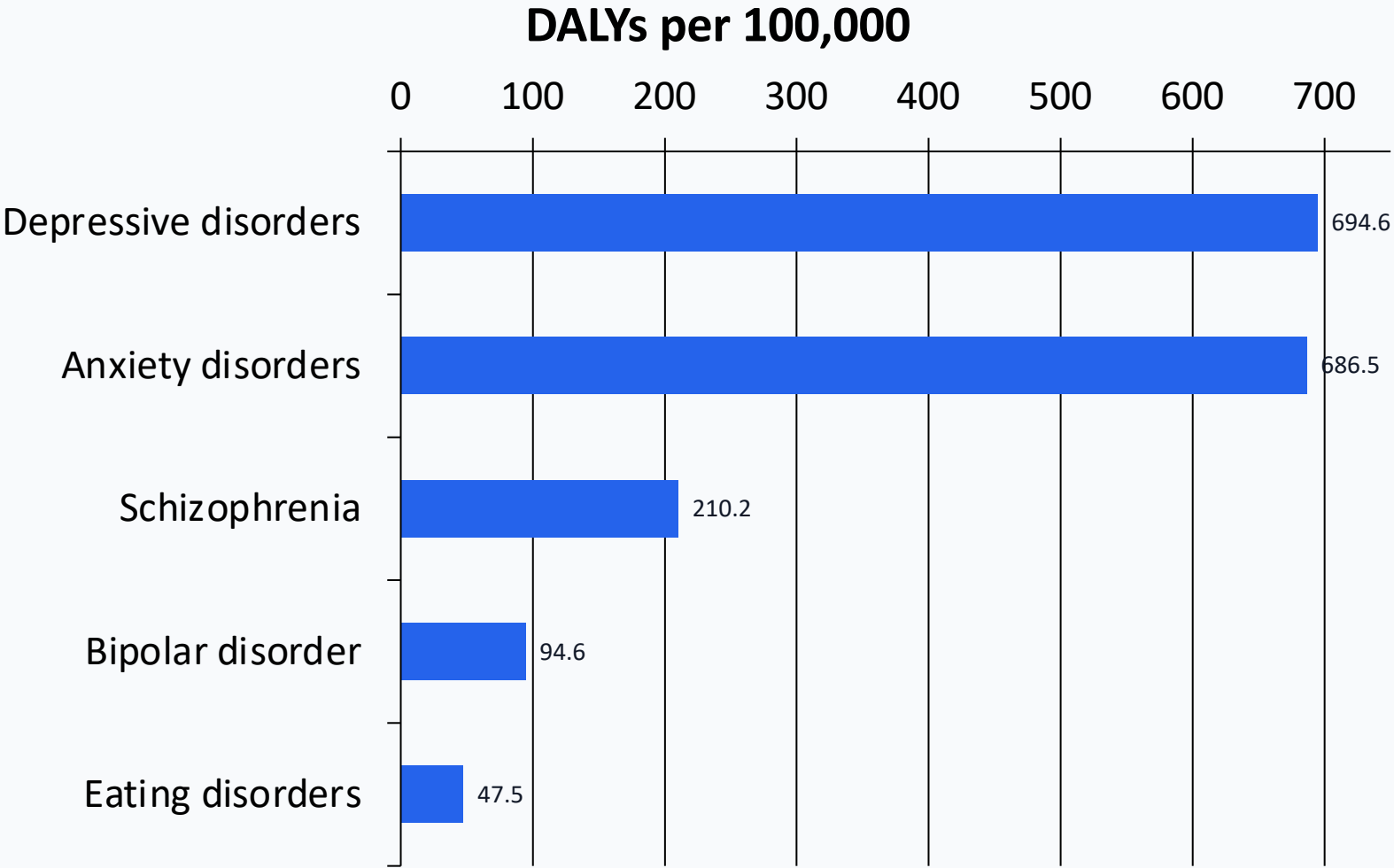
Women

1 in 5

Men

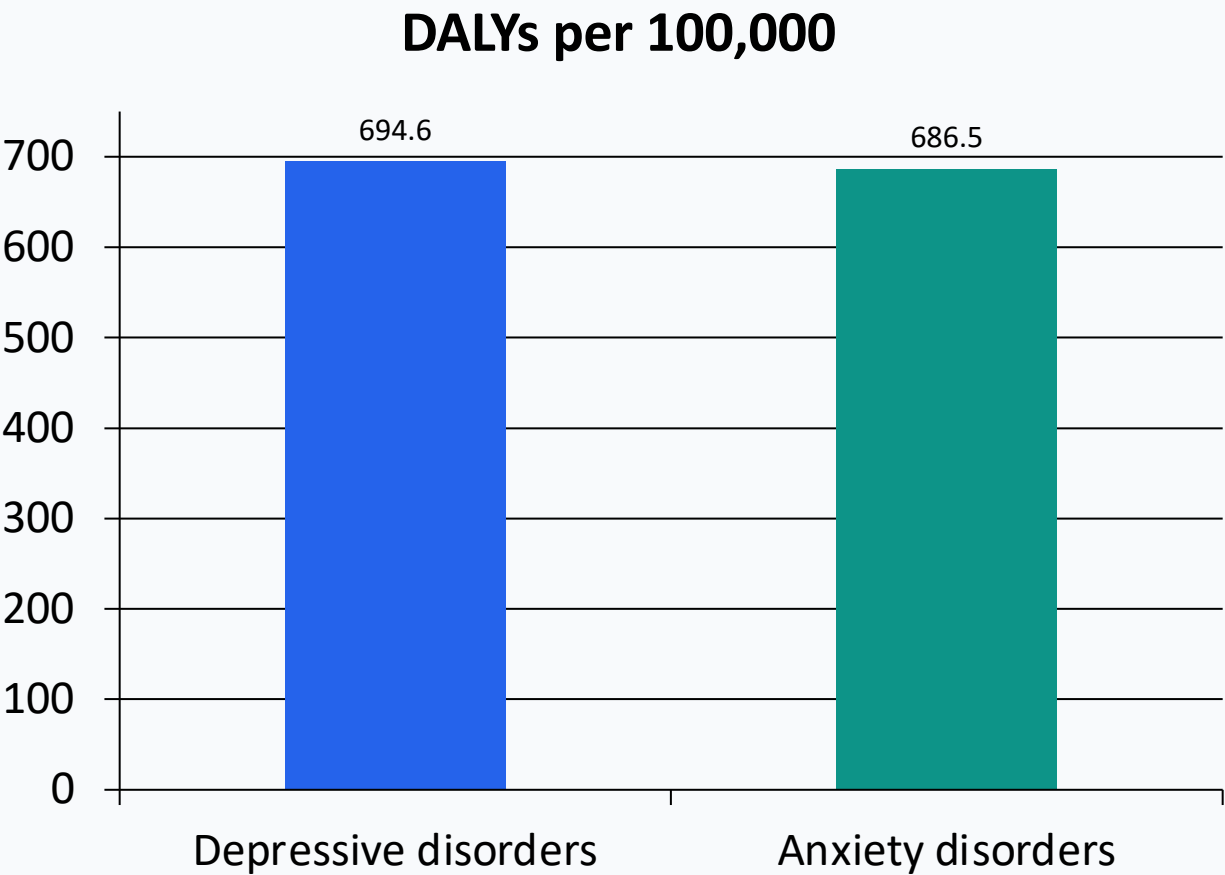
Mental health conditions range from common disorders (depression, anxiety) to less common but severe disorders (schizophrenia, bipolar disorder). Mental illnesses and substance use disorders affect hundreds of millions globally. Risk factors include genetic predisposition and environmental stressors. Scale and heterogeneity make mental health a core policy priority.

Depression and Anxiety Dominate: ~695 and ~687 DALYs per 100K Respec



Central finding
Depressive and anxiety disorders contribute the overwhelming share of mental-health DALYs.
Figure reference: burden-by-category (figure_1).

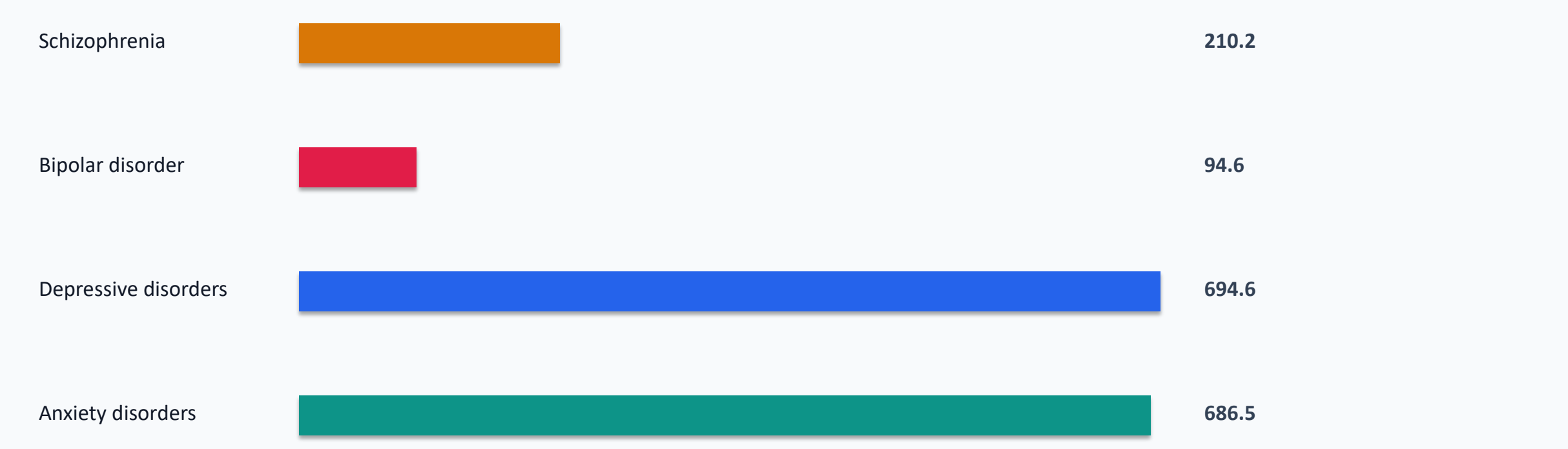
Depression and Anxiety Carry Nearly Equal Disease Burden Worldwide



Near-parity in burden

Difference is only 8.1 DALYs per 100,000.
Together, these two categories dwarf
schizophrenia, bipolar disorder, and eating
disorders.

Schizophrenia and Bipolar Disorder: Lower Prevalence but Severe Individuals



Population-level DALYs are lower for schizophrenia and bipolar disorder, but both are severe and disabling conditions requiring sustained specialized services.

Treatment Is Available but Often Lacking or Poor Quality



Stigma

Many people are uncomfortable disclosing symptoms to professionals or acquaintances.



Access gaps

Treatment is often lacking, especially in low- and middle-income countries.



Quality gaps

Even where care exists, it may be of poor quality.



Underreporting

Disclosure barriers make true prevalence difficult to estimate; burden may be underestimated.

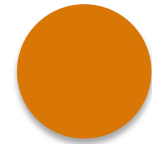
How People Cope: From Medication to Faith to Family Conversations



Prescribed medication



Religious or spiritual activities



Talking with friends or family

Coping responses vary substantially across countries and cultures, underscoring the need for context-specific policy design (figure_2).

Key Takeaways: Prioritizing Depression, Anxiety, and Closing the Treatment Gap

Depression and anxiety together represent the overwhelming majority of mental health DALYs.

Schizophrenia and bipolar disorder carry disproportionate individual burden despite lower population-level DALYs.

Treatment exists, but access and quality remain inadequate in many settings.

Stigma suppresses both help-seeking and accurate prevalence measurement.

Policy should scale evidence-based interventions for common disorders while maintaining specialized care for severe conditions.